Dr Parveen Gupta, MBBS, MD. Reg: HN010770.
Consultant Medical Specialist
I am Dr. Parveen Gupta, an internal medicine specialists and have passed my MBBS and MD at AIIMS and have forty years of experience in clinical medicine.

Patient Details: Mrs SK , 68 years old lady from Andheri East in Mumbai

Second Opinion sought by: daughter of the patient on 28th November 2020 through Telemedicine.

Summary of Information / documents provided : My mother is a 68 year old lady with hypertension and diabetic kidney disease and on Maintenance Haemodialysis for three years. Recently she has marked drop her in haemoglobin level for which she was admitted and investigated. I am enclosing the hospital records and investigation reports.

Query and Concerns of the Patient Team:
1. Kindly advise me so that we may arrive at a diagnosis to explain her anaemia.
2. How should we treat her anaemia

Expert Opinion:
I went through the records submitted by you regarding the diagnostic difficulties regarding your mother. She has severe anaemia despite regular Erythropoetin injections and Iron administration. She has undergone quite a few tests already of which the salient ones are listed with explanation:
1. Bone marrow biopsy and aspiration revealed Red cell Aplasia with few Erythroid precursor cells. The Myeloid series (that make white blood cells) and the Thromboblasts (which are Precursors of Platelets) are normal. There is no evidence of Malignancy in the bone marrow- which can displace the Red cells manufacturing percousors and cause anaemia.
2. Kappa and Lambda Light chain levels are high and there is some elevation of IgG kappa band on electrophoresis, but, since her bone marrow aspiration and bone biopsy show no increase in plasma cells, there are no suggestive of multiple myeloma. (multiple myeloma is a cause of red cell aplasia)
3. She has been investigated for Parvo virus infection which is negative. (parvo virus that normally cause 'fifth disease' is a cause of red cell aplasia)
4. Her ferritin levels are high indicating iron overload/inflammation. It may occur due to repeated blood transfusions or Iron injections.
5. There is no evidence of Haemolysis - which is breakdown of red blood cells - that can cause rapid fall in haemoglobin levels

Recommendations:

The following should be tested to look for other causes of red cell aplasia:
1. Flow cytometery for C55 and C59 to rule out aplastic phase of Paroxysmal Nocturnal Haemoglobinuria
2. Antibodies against erythropoietin may also cause Pure red cell aplasia
3. She may also be tested for Epstein Barr virus

Meanwhile she may be given an alternative for erythropoetin, such as Darbepoetin Alpha, to care of any possibility of anaemia due to erythropoetin antibodies.

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References:

1. https://emedicine.medscape.com/article/205695-overview

2. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6142432/

3. https://rarediseases.org/rare-diseases/pure-red-cell-aplasia-acquired/

E signed: Dr Parveen Gupta, MD

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Sample Second Opinion Orthopaedic case

Date:

Name of the Doctor with his degrees, affiliation and Registration

Patient details:

Opinion sought by:

Report based on the following information submitted (Links to records)
Brief history submitted by the patient
Previous doctor's prescriptions
Reports including imaging reports
Interaction with the patient on video consult

Brief summary of the case: *(How it happened?, what treatment done?, what investigations done?):*
Mr. so and so has fractured his left shoulder in sept 2020. An x ray done then showed a moderately displaced fracture of the lateral end of the clavicle. For this, he was treated in immobilsation for 6 weeks. After removal of the immobilization, he found that his shoulder is stiff, which is something expected. He was subsequently advised physiotherapy, but extended course of physiotherapy did not result in improvement of functions. A series of further x rays and CT scan showed that the fracture has not healed. I confirmed on video consultation that shoulder movements are severely restricted.
Comorbidity: He has no significant co-morbidity.
Diagnosis: Non-union fracture lateral end of clavicle with post-traumatic stiffness of shoulder.

Specific point for which second opinion is sought: He has been advised surgery by some local doctor, and hence the patient wants a second opinion.

My opinion: The above-mentioned fracture is notorious for non union, which occurs in nearly 40 % cases. Non-operative treatment is oaky for most patient where fracture is not widely displaced and for those who are not actively involved in sports. Unfortunately, in this case the fracture did not heal, and hence residual stiffness after immobilization could not be recovered by physiotherapy. At this stage, stiffness is bad enough to be treated by physiotherapy alone, and also attempt at physiotherapy in the presence of an unhealed fracture may not be effective.

My recommendations: The patient has a choice to persist with physiotherapy, which, in my opinion considering his active life and disability, will not be effective. Hence, I would suggest that we operate now. Before taking him up for surgery, one would need do an MRI and some x rays. Operation will consist of fixing the fracture and perform arthroscopic release of the shoulder at the same time. Patient will need extended post-op physiotherapy for 2 months. He is expected to regain 90
% shoulder function in 3 months. The usual complications associated with such an intervention are discussed with the patient. Specific complication is recurrence of infection and prolonged physiotherapy and possible manipulation of the shoulder.

References:

Signature of the Doctor